

# DS2444B — The Medical Report: A Completion Guide

*A clinical, question-by-question guide for the health professional completing the disability medical report, with fill-in templates keyed to the eligibility test.*

## For the completing health professional

This is a shorthand guide to the DIA Medical Report (DS2444B). It maps each field to what the adjudication turns on, and offers **fill-in templates** keyed to the eligibility test. Templates are scaffolding only: complete every bracket from your own examination and record, and delete any clause that does not fit this patient. Your signature certifies your clinical opinion, so the report must reflect it. The official form is at [open.alberta.ca](https://open.alberta.ca).

## The test the report must speak to

AISH eligibility: a **severe and permanent** impairment that **substantially limits the ability to earn a livelihood**. Document the **limitation** and its **permanence** — not the accommodations that would let the patient work. “Accommodation” language reads as *capable with support* (the ADAP finding). Where clinically true, state that no accommodation renders competitive employment reliable or sustainable. Unpredictability and inability to self-sustain are often the strongest findings: an employer can adapt a workstation; none can adapt to unpredictable incapacity.

## Relationship With Applicant (Q1–Q4)

Q1: confirm Alberta registration; give discipline and registration number. Q2–Q4: treating duration, last contact, and frequency. Longer, regular treating relationships carry evidentiary weight — state them plainly (e.g., “primary provider since [yyyy], reviewed [monthly]”). If you are a specialist seeing the patient episodically, say so; it frames the scope of your opinion.

## Diagnosis and Medical History (Q5–Q10)

### Q5. Diagnoses + DIA/DSM codes + onset

List all conditions, primary and secondary, with codes and onset dates. Early or congenital onset dates support permanence — record them precisely. Include comorbidities that compound function even if individually sub-severe; the cumulative picture is what is adjudicated.

### Q6. Diagnosis detail + specific symptoms

Etiology, classification, stage/grade, and the specific symptoms per diagnosis.

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[Condition] — [etiology/classification]. Symptoms this patient experiences: [list]. Functional consequence: [what the symptoms prevent — e.g., sustained concentration, reliable attendance, self-initiation of tasks]. Onset [mm-yyyy]; course [static / progressive / relapsing].

**Q7. Medical history + chronology; “delineate impairment, duration, restriction”**

This is the central narrative field. Tie each condition to a duration and a functional restriction, and close on permanence and impact on earning capacity.

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[Patient] has a [X]-year documented history of [condition(s)], first presenting [mm-yyyy], with [stable / progressive] course despite [treatments tried]. The impairment restricts [functions]. These restrictions are [continuous / unpredictable and recurrent] and are not resolved by workplace accommodation, because [reason — e.g., the limiting factor is unpredictable incapacity, not the work environment]. In my opinion the condition substantially limits this patient’s ability to earn a livelihood on a reliable, sustained basis.

*Attach supporting reports and check “Yes” at Q7. If unavailable, explain briefly rather than leaving blank.*

**Q8–Q10. Referrals, other evidence, admissions**

Q8: list specialist consultations and attach reports — corroborating specialist evidence strengthens the file.

Q9: attach diagnostics, imaging, labs. Q10: list relevant hospital admissions with dates and reasons. Where documents exist, attaching them pre-empts a request for further information later.

**Degree of Impairment — Activities of Daily Living (Q11–Q40)**

**How to rate (0–3), and why the describe boxes matter**

Rate each item to the true clinical picture: 0 none, 1 mild, 2 moderate, 3 severe/unable. Do not under-rate to appear conservative — an accurate 2 or 3, with a one-line note in the “describe limitation” column, carries the file. Adjudication weighs the **cumulative** burden across domains, so complete the describe boxes wherever a rating is 2 or 3, and note variability (“3 on flare days, 1 between”) rather than averaging it away.

**Q40. Describe symptoms impacting cumulative ADL impairment (free text)**

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Across mobility, cognition, personal care, and independent living, this patient’s cumulative impairment is [moderate–severe]. Key drivers: [items rated 2–3]. On [flare / bad] days, [patient] requires [assistance/supervision] for [tasks]. The combined effect is that [patient] cannot independently maintain the routine and reliability that sustained employment requires.

**Level of Impairment — Mental Health / IEWS (Q41–Q68)**

Same rating discipline (0–3) across mood, thought/perception, cognition, behaviour, orientation, speech, somatic, and safety. Note the fields that most often carry these files:

- Q50–52 attention / concentration / comprehension deficits.
- Q53 executive function deficits (self-regulation, planning, organization) — pivotal for acquired brain injury and neurodevelopmental profiles; rate and describe.
- Q57 memory (long / short / working) — specify which.
- Q60 amotivation, Q59 impulse control, Q43 emotional dysregulation — describe the functional cost.

Q67 (suicidality) is a clinical safety field — complete it from your assessment as you would any risk documentation. No template is offered for it.

### Q68. Describe symptoms impacting cumulative IEWS impairment (free text)

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Cumulative mental-health impairment is [moderate–severe], driven by [items rated 2–3]. [Executive dysfunction / cognitive deficits / mood instability] mean [patient] cannot reliably [initiate and sequence tasks / sustain attention / regulate under ordinary workplace demand] without continuous external structure. These deficits are [longstanding / organic] and persist [despite treatment]. They are not correctable by accommodation, as they affect the capacity to self-organize and self-sustain that any employment presupposes.

## Remedial Therapy (Q69–Q74)

### **The permanence gate lives here**

Denials often turn on the theory that more therapy *might* improve the condition, so it is not yet “permanent.” Answer these fields so the record closes that gate where it is clinically closed: treatments tried and exhausted, and why further treatment is not expected to restore work capacity. If a treatment is nominally available but inaccessible in a clinically relevant timeframe, say so — therapy the patient cannot access for [12–16+ months] is not meaningfully available.

### Q69–Q70. Medications + effect on functioning; treatment history + results

List medications and their functional effect (including sedation or cognitive side effects that themselves limit work). Give treatment history with timeframes and outcomes.

### Q71. Current plan / why none, if none

State the current plan, when initiated, and review frequency — or, if options are exhausted, say so explicitly.

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Current management: [plan], initiated [mm-yyyy], reviewed [interval]. Available interventions have been [tried / exhausted] with [partial / no] functional gain. [Remaining options are limited to symptom management, not restoration of work capacity. / Further options are inaccessible within a clinically relevant timeframe: [waitlist / access barrier].]

### Q72–Q73. Treatment compliance

Answer truthfully. If “No,” explain — an unexplained “non-compliant” can sink an otherwise strong file. Legitimate reasons belong on the record.

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Adherence is limited by [cost / side-effect intolerance / access / the disability itself — e.g., executive dysfunction impairing medication management]. This is a consequence of the condition and circumstances, not refusal of care.

### Q74. Further treatment anticipated?

If further treatment is expected to help, describe it and the timeframe. If it is not expected to restore the capacity for sustained work, state that plainly — this directly supports the prognosis.

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No further treatment is anticipated that would be expected to restore this patient’s capacity for reliable, sustained employment. Management is directed at [symptom control / stability], not functional recovery.

## Prognosis (Q75) — the determinative field

Four options: Temporary, Episodic, Permanent, Undetermined. Where clinically supported, **Permanent — not expected to change or improve over time with treatment** is the finding the AISH determination rests on. For episodic conditions, use Episodic and quantify frequency, length, severity, and total duration — unpredictable recurrence is itself a barrier to employment. Avoid “Undetermined” if the record supports permanence; an ambiguous prognosis defaults the file toward the lower program.

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Permanent. [Condition(s)] are longstanding and not expected to improve with further treatment. Prognosis for return to reliable, sustained employment is poor. [For episodic: episodes occur [frequency], last [duration], severity [level], onset unpredictable; between episodes the patient [residual limitation].]

## Additional Comments (Q76)

The closing synthesis. Draw the domains together into one statement of cumulative impairment, permanence, and impact on the ability to earn a livelihood.

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In summary, [patient] lives with [severe and permanent condition(s)] producing cumulative impairment across [physical / cognitive / mental-health] domains. The limitations are [continuous / unpredictable], not correctable by workplace accommodation, and not expected to improve with treatment. In my clinical opinion, [patient] is substantially limited in the ability to earn a livelihood on any reliable, sustained basis.

## Records to Draw On, and Certification

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### **Netcare is not the full record**

Alberta Netcare holds labs, imaging, dispensed medications, and hospital records — not community clinic office notes. Where relevant history sits outside Netcare, draw on the clinic chart, pharmacy record, and specialist correspondence so the report is complete rather than partial.

Certification (final page): completing the report certifies it contains your clinical findings and medical opinion. Provide name, CPSA/CRNA number, contact details, date, and signature.

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### **The guardrail that protects everyone**

Every template above is completed to the true clinical picture. Accuracy is the strongest position and the one that withstands review — an overstated report can be challenged and can cost the patient the case they truly have. Document the real impairment fully, in the language the determination weighs.